

COUNSEL MEMORANDUM*Proposed Tyler Alternate Delivery Site Operating Bridge***To:** Profound Hospice Care's Legal Counsel**Through:** Patrick Ayeni**From:** Geoff Schackmann, on behalf of Tyler Hospice Hold, LLC and its future operating subsidiary (intended DBA: Azalea Hospice)**Date:** August 6, 2026**Re:** Counsel review of proposed Tyler ADS structure and implementation documents**Status:** *Agreement in principle; subject to advisor review, regulatory approval, and definitive documentation*

Counsel review requested. This memorandum records the business framework the parties intend to evaluate. It is not a contract, legal opinion, commitment to furnish or bill for patient care, or authorization to use Profound's identifiers. No Medicare-reimbursed hospice services should be furnished from the Tyler location until counsel and the responsible regulatory authorities confirm that all required approvals and operational conditions are in place.

1. Purpose and current status

Profound Hospice Care ("Profound") and Tyler Hospice Hold, LLC and its future operating subsidiary (intended DBA: Azalea Hospice) have reached an agreement in principle to pursue a temporary Tyler alternate delivery site ("ADS") under Profound's existing hospice provider platform. The parties intend to use their August 10, 2026 in-person working session to confirm the operating framework, assign implementation responsibilities, and direct counsel to prepare definitive documents.

The proposed relationship is a sponsored provider arrangement—not a lease, sale, assignment, or informal use of Profound's Medicare billing privileges, NPI, EIN, CCN, or other credentials. Profound would remain the licensed and enrolled hospice provider of record for all Tyler patients admitted under the arrangement. Tyler Hospice Hold, LLC and its future operating subsidiary would fund the Tyler expansion and provide defined nonclinical infrastructure and support, but it would not replace Profound's clinical, regulatory, or billing authority.

The parties' target is operational readiness by the end of August 2026. That target is aspirational and must remain subordinate to all Texas HHSC, CMS, Medicare Administrative Contractor ("MAC"), accreditation, payer, employment, vendor, and other required approvals.

2. Proposed operating structure**2.1 Profound's role as provider of record**

Profound would establish and operate the Tyler ADS as part of its hospice, subject to state and federal approval. Professional and administrative authority would trace to Profound in both the documents and actual practice. Profound would retain final authority over:

- patient admission, election, eligibility, certification, recertification, discharge, and transfer decisions;
- plans of care, interdisciplinary-group functions, clinical policies, QAPI, complaints, surveys, audits, and corrective action;
- clinical hiring standards, credentials, supervision, discipline, and termination;
- claims submission, billing edits, refunds, overpayments, hospice cap matters, appeals, and payer communications;
- payer receipts and the bank account into which those receipts are deposited;

- access to and control of the authoritative clinical and billing records; and
- an immediate right to pause or stop Tyler admissions when Profound reasonably determines that patient safety, documentation, compliance, staffing, funding, or billing integrity is at risk.

Texas guidance states that an ADS must be part of the hospice, share administration, supervision, and services with the parent agency, and remain under the parent's continuing management. It also calls for documented monthly onsite supervision, parent approval of ADS policies, and maintenance of the original active clinical record at the ADS. Counsel should confirm how these requirements will be reflected in governance, policies, staffing, and record architecture.

2.2 Azalea's role

Tyler Hospice Hold, LLC and its future operating subsidiary would fund all Tyler startup costs, working-capital needs, operating losses, approved direct costs, and required reserves. Subject to Profound's reserved authority and approved budget, Azalea would provide or coordinate the Tyler office, recruiting support, technology, local market development, finance and analytics, implementation project management, and other defined nonclinical services. Azalea would also continue developing its separate future hospice license and Medicare enrollment.

The definitive documents should distinguish recommendations and day-to-day support from provider-level control. If Azalea or an affiliated entity will exercise operational or day-to-day management functions that create Medicare enrollment disclosure obligations, counsel should identify the required CMS-855A/PECOS disclosures before launch.

3. Staffing, employment, and payroll

Recommended base case. Profound should be the W-2 employer of the core Tyler clinical workforce and any other positions that law, regulation, payer rules, or Profound policy require the hospice to employ. Azalea should prefund those costs, while Profound maintains one authoritative payroll-tax filing process. Azalea may separately employ clearly nonclinical personnel.

The parties initially considered using a separate Gusto payroll configuration for Tyler. Counsel and Profound's payroll-tax advisor should determine whether that can be implemented under Profound's EIN without duplicate or inconsistent employment-tax filings, benefit-plan complications, workers' compensation gaps, or ambiguity over the employer of record. A separate payroll user interface or cost center may be feasible; a second independent tax filer under the same EIN should not be assumed.

The staffing documents should address:

- which positions Profound must employ, including the administrator and substantially all nursing, medical social work, and counseling personnel;
- offer letters, wage-and-hour compliance, payroll calendar, benefits, leave, workers' compensation, unemployment, background checks, credentialing, and mandatory training;
- Azalea's prefunding deadline, minimum payroll reserve, reconciliation rights, and consequences of a funding shortfall;
- Profound's final authority over clinical hiring, supervision, discipline, and termination; and
- the process for an orderly employee transition when the bridge ends, without impairing patient care or creating an automatic transfer obligation.

4. Compensation and funding

4.1 Fixed fee and direct costs

The current business proposal is a fixed payment of \$7,500 per month to Profound, plus reimbursement of documented, approved direct Tyler costs at cost and without markup. The fixed fee is intended to compensate Profound for real implementation, provider oversight, administrative support, compliance, billing, and related services—not for referrals, admissions, patient volume, or access to billing credentials.

The first \$7,500 payment is targeted for September 1, 2026, triggered by execution of the agreed non-binding framework and formal commencement of implementation. Counsel should resolve an important drafting point: a wholly non-binding term sheet ordinarily does not create an enforceable payment obligation. If the parties intend the September 1 payment to be binding, counsel should either (a) designate the payment and related implementation provisions as expressly binding within an otherwise non-binding term sheet, or (b) prepare a short binding implementation letter that operates until the definitive agreements become effective.

Counsel should review the fee and reimbursement methodology under the federal Anti-Kickback Statute, applicable Texas law, cost-reporting rules, and any payer requirements. The parties should obtain and retain appropriate fair-market-value and commercial-reasonableness support. The federal personal-services and management-contract safe harbor generally requires a signed writing, a term of at least one year, specified services, and compensation methodology set in advance, consistent with fair market value, and not determined by referral volume or value. The transaction should not be characterized as a safe-harbor-compliant arrangement without counsel's analysis of the actual services, parties, and payment direction.

4.2 Transition consideration—Patrick's election

In addition to the monthly fee and direct-cost reimbursement, Patrick will choose between two proposed transition alternatives before the definitive documents are completed:

1. \$25,000 transition cash payment, with the timing, conditions, reconciliation, and any holdback specified in the definitive agreement; or
2. \$50,000 convertible note, with issuer, holder, interest, maturity, security, conversion triggers, valuation methodology, discount or cap, maximum ownership, transfer restrictions, and repayment rights to be developed by counsel.

The current proposal includes no exclusivity covenant. The transition consideration must not be conditioned on patient referrals, admissions, continued use of Profound's credentials, or any volume or value of federal healthcare program business. If a note is selected, counsel should also address securities, tax, corporate-approval, ownership-disclosure, and provider-enrollment implications and decide whether the holder should be Patrick, Profound, or another permitted entity.

5. Clinical records, FireNote, and information governance

Azalea proposes using FireNote for the Tyler patient population because of its documentation workflow, productivity features, and perceived documentation-integrity strengths. FireNote may be used only if Profound approves the configuration and retains complete provider-level control. At a minimum, diligence should confirm that FireNote can:

- segregate Tyler patients while supporting Profound's complete hospice record and reporting obligations;
- give Profound full administrator, clinical oversight, audit, export, and termination access;
- support claims under Profound's properly approved enrollment and billing configuration;
- retain and produce the complete record in required formats and timeframes;
- integrate with Profound's QAPI, compliance, billing, and records-retention processes; and
- satisfy HIPAA security, breach-response, business-associate, user-access, logging, backup, and disaster-recovery requirements.

FireNote must not operate as an uncontrolled "shadow" record. The definitive record-of-truth, interface, amendment, retention, ownership, access, export, and claims-workflow rules should be documented in an EMR and data-governance schedule. Profound's current Hospice Tools environment should be evaluated alongside FireNote so the parties understand whether they will use one platform, a controlled interface, or a migration process. **A separate clinical demonstration involving Profound's clinical representative and the Tyler clinical team is recommended before a platform decision.**

6. DME Express and other vendors

DME Express is currently contracted with Azalea. That contract alone should not be treated as authority to furnish DME for Profound patients. Before launch, counsel and Profound should determine whether Profound will enter a new agreement with DME Express, approve a legally permissible assignment or subcontract, or select another structure. Any arrangement should preserve continuity for patients while keeping DME under Profound's plans of care, vendor oversight, documentation, service-level, billing, privacy, insurance, and indemnification requirements.

The same approval discipline should apply to pharmacy, laboratory, facility, after-hours, medical-director, billing, background-screening, and other patient-care vendors. Vendor preference may be considered, but Profound must retain final approval and oversight.

7. Enrollment, licensure, and launch conditions

No Medicare-reimbursed service should be furnished from Tyler until the location is established and operated as an approved ADS of Profound, rather than as an independent hospice using Profound's identifiers. The parties should confirm the following minimum conditions in writing before the first Medicare bill:

- Texas HHSC/TULIP approval of the ADS application for 13387 North Highway 69, Tyler, Texas, parent-license relationship, administrator and supervising-nurse structure, and any required license or service-area updates;
- Tyler's inclusion within Profound's licensed service area and addition of the location to Profound's CMS-855A/PECOS practice-location record, including the effective date and hospice-treatment designation;
- completion of the MAC, HHSC/state-agency, CMS, iQIES, and—where applicable—accreditation review, survey, or site-visit steps, with written confirmation of the effective approval date;
- payer-specific Medicaid, managed-care, and commercial enrollment or contract updates, together with EFT, electronic-remittance, clearinghouse, and claims-routing configuration for each payer the parties intend to bill;
- local occupancy and signage compliance, active-record controls, emergency preparedness, communications, 24-hour service coverage, and the parent hospice's documented monthly supervisory-visit process; and
- tested EMR, claims, payroll, banking, staffing, vendor, privacy, QAPI, complaint, and compliance workflows, with Profound retaining final clinical, regulatory, billing, and financial oversight.

CMS requires institutional providers to use PECOS/CMS-855A for enrollment changes and to report a change in practice location within 30 days. Texas HHSC requires CMS approval of the ADS before Medicare-reimbursed hospice services are furnished from the location. The implementation team should confirm the sequence, responsible party, evidence, and effective date directly with HHSC, Palmetto GBA, CMS/state survey channels, and any accreditor. Submission of an application is not authorization to bill, and no new CCN, NPI, or EIN for the ADS should be assumed without written MAC confirmation.

7.1 ADS go/no-go approval matrix

Workstream	Minimum evidence before launch	Responsible owner	Status / effective date
Texas ADS license	TULIP approval and issued ADS license for 13387 North Highway 69, Tyler, Texas, parent-license link, service-area confirmation.	Profound / HHSC	Pending / date issued
Medicare enrollment	CMS-855A/PECOS add-location submission accepted; MAC/CMS approval and effective date received.	Profound authorized official / Palmetto GBA	Pending / effective date
Survey or accreditation	Written determination of whether HHSC survey, CMS/state survey, accreditor review, or site visit is required; all findings closed.	Profound / HHSC / accreditor	Pending / clearance date
Provider control	Organizational chart, written authority matrix, monthly supervisory-visit protocol, records and QAPI ownership.	Profound administrator	Pending / approval date
Clinical readiness	Qualified staff, 24-hour coverage, plans of care, IDT process, emergency preparedness, complaint and incident workflows.	Profound clinical leadership	Pending / readiness date
Payer and claims	Medicaid/MCO/commercial updates as applicable; EFT/ERA, clearinghouse, claim routing, NOE/NOTR and billing tests.	Profound billing / payer teams	Pending / test date
Systems and vendors	EMR, DME, pharmacy, lab, privacy, access, backup, and vendor oversight workflows tested and approved.	Joint implementation team; Profound final approval	Pending / sign-off date
Go/no-go authorization	Signed launch certificate confirming every applicable row is complete and the billing effective date has arrived.	Patrick / Profound authorized official	Not authorized until signed

8. Patient, billing, cash, and reconciliation controls

Profound would control payer enrollment, claims submission, payer receipts, and the bank account into which Tyler collections are deposited. The definitive documents should establish a Tyler cost center and a transparent monthly reconciliation package showing patient-level revenue attribution, billed and unbilled claims, remittances, denials, accounts receivable, direct costs, payroll, reserves, refunds, overpayments, cap allocation, management fees, working-capital advances, and true-up calculations.

All Tyler patients admitted during the bridge would be Profound patients and would execute Profound-compliant elections and other required documents. At the end of the bridge, no patient should be automatically “moved” to Azalea. Any change of designated hospice must respect beneficiary choice and applicable transfer

documentation and continuity-of-care requirements. Profound should remain responsible to regulators and payers for claims submitted under its provider agreement, subject to contractual allocation, indemnification, reserves, records cooperation, and claims-runout provisions between the parties.

9. Proposed document architecture

Counsel may consolidate these items into one definitive agreement with schedules or use a coordinated agreement suite. The business team anticipates the following components:

1. non-binding term sheet, with any intended binding provisions clearly identified;
2. ADS operating and administrative-services agreement for a 12-month initial term, with one additional 12-month extension only by mutual written agreement;
3. working-capital prefunding, direct-cost reimbursement, reserve, accounting, reconciliation, and audit-rights schedule;
4. clinical staffing, employment, supervision, payroll, and transition schedule;
5. RACI and implementation plan with named role owners and decision rights;
6. EMR, HIPAA/business-associate, cybersecurity, record-of-truth, data-access, retention, and transition schedule;
7. approved-vendor schedules, including DME Express if approved by Profound;
8. insurance, indemnification, overpayment, hospice-cap, claims-runout, audit-cooperation, and survival provisions;
9. termination, stop-admission, cure, emergency action, patient-continuity, and transition provisions; and
10. either a transition-payment provision or separate convertible-note documents, depending on Patrick's election.

10. Principal questions for counsel

1. Does the proposed allocation of authority accurately establish Profound as the provider furnishing Tyler services, rather than permitting another entity to use Profound's billing privileges?
2. What TULIP, CMS-855A/PECOS, MAC, survey, accreditation, payer, and service-area approvals are required, in what sequence, and with what effective dates?
3. Which Tyler positions must be Profound employees, and what employment/payroll structure avoids ambiguity, duplicate tax filings, and benefits or insurance gaps?
4. Does Azalea's proposed scope create a reportable managing-organization or other enrollment disclosure, and how should reserved powers be documented and practiced?
5. How should the \$7,500 fixed fee, direct-cost reimbursement, September 1 implementation payment, and transition alternative be structured and supported for fair-market-value, commercial-reasonableness, Anti-Kickback Statute, Texas-law, tax, and cost-reporting purposes?
6. If the framework remains non-binding, what narrow binding instrument should govern implementation payments, confidentiality, expenses, data access, and conduct before the definitive agreement is signed?
7. What conditions must FireNote meet to serve as the authoritative Tyler record, and what agreements are required with FireNote, Hospice Tools, clearinghouses, and other data vendors?
8. What contracting path is required for DME Express and other vendors, and how should continuity, claims, HIPAA, insurance, and indemnification be handled?
9. What insurance, indemnity, reserve, guaranty, limitation-of-liability, audit, overpayment, cap, tax, and claims-runout protections are appropriate given Profound's direct provider exposure?

10. How should termination and patient transition be structured to preserve beneficiary choice, continuity of care, records access, receivables, and regulatory accountability?
11. Would the convertible-note option create securities, tax, corporate, provider-enrollment, ownership-disclosure, or change-of-control issues, and who should be the issuer and holder?
12. What representations, diligence certificates, compliance covenants, audit rights, and launch conditions should be satisfied before Profound admits the first Tyler patient?

11. Working timeline and conditions

Target	Action / condition
August 10	Confirm business points, Patrick's transition-option process, decision rights, document architecture, role owners, and launch workplan.
Immediately after August 10	Complete regulatory, payroll, EMR, vendor, insurance, finance, and compliance diligence; open TULIP/PECOS/MAC/accreditor workstreams as counsel directs.
Before September 1	Execute the non-binding framework and any narrow binding implementation letter or binding term-sheet provisions; begin formal implementation.
September 1	First \$7,500 implementation payment, if the agreed trigger documents are executed and implementation has formally commenced.
Before first admission	Obtain and document every required state, federal, payer, staffing, vendor, EMR, billing, and operational approval. No launch solely because a target date has arrived.
After launch	Monthly supervisory visit, governance review, compliance and QAPI monitoring, financial reconciliation, claims review, reserve testing, and implementation-status reporting.

12. Requested counsel response

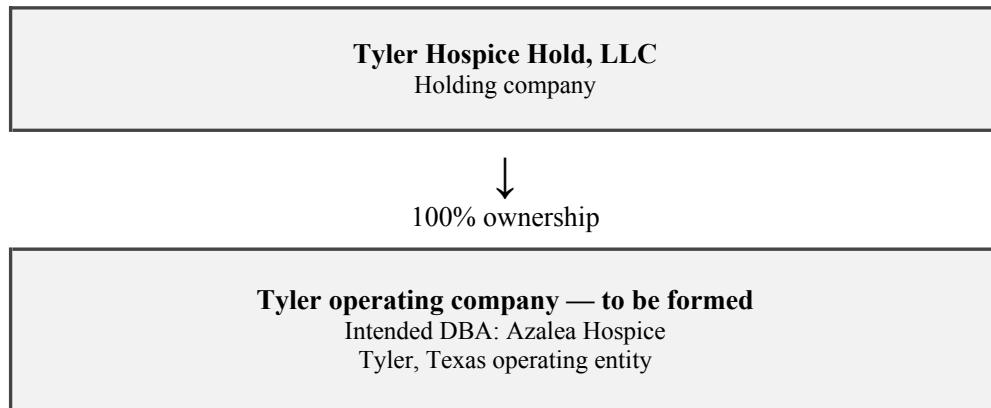
Please redline this memorandum or provide a short issue list identifying: (1) any structural element that should not proceed as proposed; (2) required changes to preserve Profound's provider control; (3) approvals and conditions that control the launch sequence; (4) documents and diligence required before implementation; and (5) the recommended form of the September 1 payment obligation. The business team's objective is to move promptly, but no schedule should override legal, regulatory, clinical, or billing requirements.

Non-binding and subject to counsel. Except for any provision that the parties later expressly designate as binding in a signed writing, this memorandum is solely a discussion draft. It creates no partnership, joint venture, agency, employment, referral, patient-transfer, license-use, billing, payment, exclusivity, or other obligation. Qualified healthcare regulatory, employment, tax, securities, and transactional counsel should review the final structure and documents.

Appendix A. Selected primary authorities for counsel's review

42 C.F.R. § 418.100 — Hospice organization and administration of services
42 C.F.R. § 418.64 — Core services and hospice-employment requirements
42 C.F.R. § 418.104 — Clinical records
42 C.F.R. § 424.550 — Prohibitions on sale or transfer of billing privileges
42 C.F.R. § 1001.952(d) — Personal-services and management-contract safe harbor
Texas HHSC — Hospice Alternate Delivery Sites
CMS — Enrollment Applications / CMS-855A and PECOS
CMS — Institutional Provider Enrollment Guide

Appendix B. Entity structure for drafting



Drafting note. The Tyler operating company has been identified and is currently undergoing diligence, but it does not yet exist as a formed subsidiary. Tyler Hospice Hold, LLC is intended to own 100% of that future subsidiary, and “Azalea Hospice” is the intended DBA/trade name of the operating company—not a separate existing legal entity.